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In philosophy, the *Lottery Paradox* asks: **If you know one ticket will win, but each ticket is unlikely to win, is it rational to believe each ticket will lose?**

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It's a cautionary tale about how we misjudge low-probability, high-impact events.

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In healthcare, self-funded employers fall into the same trap every day, especially with organ transplants, gene therapies, and CAR-T treatments. The cost of this thinking? Seven-figure claims that blow past stop-loss.

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"It's Unlikely to Happen to Us..."

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Employers aren't wrong. Each catastrophic event is rare. But here's the paradox: **Individually unlikely ≠ Collectively avoidable.**

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With the pace of medical innovation, the probability that *something* high-cost will hit your plan is not "if," it's "when."

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You're not betting on a single ticket. You're holding the *entire* raffle.

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For Self-Funded Employers and Their Advisors: This Is Not Theoretical

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If you're a benefits consultant or broker, you've likely heard "These treatments are edge cases, so we don't need to prioritize them."

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The data says otherwise:

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- 69,000+ U.S. transplants each year

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- CAR-T therapy use growing rapidly, especially in younger cancer patients

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- Gene therapies accelerating, with multiple \$2–3M FDA-approved treatments already in play

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For groups with 5,000+ members, these cases are no longer hypothetical. They're actuarial certainties.

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When Rare Hits, It Hits Hard

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High-cost medical events:

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- Routinely blow through stop-loss budgets

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- Trigger out-of-network costs (even at “in-network” hospitals)

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- Are often managed via carve-out networks with poor transparency

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- Can vary by \$200K–\$1M+ depending on routing and contract type

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At HealthRate, we see:

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- The *same* hospital billing wildly different rates for the *same* transplant based on referral source

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- Gene therapies marked up by hundreds of thousands (sometimes millions)

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- Specialty networks charging 20–40% more than direct contracts for identical cases

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How Advisors Use HealthRate to Win on Risk Management

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We help you intervene before the bill arrives by:

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- Flattening internal provider rate variance across networks

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- Modeling stop-loss exposure *before* the claim hits

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- Comparing clinical outcomes and reimbursement rates

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- Identify opportunities for high-quality, lower cost services

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This Is an Advisory Moment

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For brokers and consultants, managing million-dollar claim risk isn't optional—it's where strategic value is proven.

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With HealthRate, you shift the client conversation from “*That'll probably never happen*” to “*Here's exactly how we're protecting your plan from it.*”

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Bottom Line: The Paradox Holds

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Each event may be rare. But *some* event will happen.

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The only irrational move? Believing otherwise.

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Ready to protect your plan from the next million-dollar surprise?

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If you're a benefits consultant or self-funded employer, contact us for a free exposure analysis, or preview our analytics tools in action.